HIVES

What it is

- Raised, swollen, intensely itchy welts that can appear anywhere on the body
- Treatment starts with antihistamines like cetirizine. For chronic hives, doctors may use omalizumab, an injection to block the antibody involved in histamine release

New treatments

- Remibrutinib is a new oral drug that blocks BTK, a protein involved in activating mast cells, which release histamine
- Barzolvolimab is an emerging treatment that targets KIT, a receptor that mast cells (immune system white blood cells) need to survive

PSORIASIS

What it is

- Thick, red patches covered with silvery-white scales that may itch, often on the elbows, knees, scalp
- An overactive immune system speeds up skin-cell growth, causing cells to build up on the surface
- Treatment includes moisturisers, steroid and vitamin-D creams; for severe cases, medicines that target immune activity

New treatments

- Deucravacitinib is an oral drug that blocks TYK2, an immune-signalling protein involved in psoriasis, helping reduce plaques and itching
- Newer biologics, such as secukinumab, ixekizumab and guselkumab, block specific immune signals to reduce redness, scaling and itching in psoriases

CHOLESTATIC PRURITUS

What it is

- Persistent, often intense, itching, usually without a visible rash
- Associated with liver or bile-duct disorders
- Treatment starts with cholestyramine, which binds bile acids in the gut

New treatments

- IBAT inhibitors are newer drugs that block the reabsorption of bile acids in the gut, helping the body eliminate more of them and potentially reducing itching

UREMIC PRURITUS

What it is

- Persistent, often intense, itching, usually without a visible rash
- Common in people with advanced kidney disease, particularly those on dialysis
- Treatment includes antihistamines, optimising dialysis and, in some patients, gabapentin or pregabalin to calm nerve signals

New treatments

- Difelikefalin is a drug developed specifically for moderate-to-severe itching in such cases. It activates itch-controlling receptors outside the brain, with fewer risks than traditional opioids

Note: *Some newer medicines are not routinely available in India but may be accessed through specialist-led, named-patient import channels*

mines to modern cures like biologic medicines, Janus kinase or JAK inhibitors and neural pathway blockers. Eczema, for instance, is treated with what are called 'interleukin (IL) antagonists' such as dupilumab and lebrikizumab, which block the specific immune signals that drive inflammation and make sensory nerves hypersensitive to itching. Nemolizumab, a monoclonal antibody, targets IL-31, a chemical messenger closely associated with the sensation of itch. Another approach targets the signalling pathways inside cells that sustain inflammation and itching. In conditions such as eczema, some of these messages become overactive, making the skin red, inflamed and itchy. JAK enzymes help carry these messages within cells. And JAK inhibitors interrupt this process, helping calm inflammation and relieve itching. These include tablets such as upadacitinib and abrocitinib, as well as a topical cream containing ruxolitinib.



"Acne itself can damage self-esteem. Fungal infections, scabies or eczema can be harder socially as people don't understand them"

—DR UPASANA CHADHA, *Psychologist, Delhi*

Then there are medicines that help reduce itching by blocking the signals that the brain interprets as an itch. When the skin is irritated or inflamed, the body releases a chemical messenger called substance P. It carries signals through the nervous system and can contribute to itching and inflammation. Neurokinin-1 (NK-1) inhibitors, such as serlopitant and tradipitant, work by blocking the receptors that substance P attaches to, helping interrupt these itch signals.

Topical creams, too, no longer rely on steroids to relieve an itch. Roflumilast and tapinarof, for instance, work to reduce skin inflammation. While roflumilast blocks PDE-4, an enzyme that helps drive inflammation, tapinarof works by activating a receptor in skin cells that helps control inflammation and strengthen the skin's natural protective barrier. "What is exciting now is that we are no longer treating every severe itch the same way," says Dr Mahajan. "Newer medicines can target the specific inflammation or immune signal driving conditions such as eczema, and some newer non-steroid creams can calm the skin without relying on corticosteroids. But these treatments only work when the diagnosis is right. They are not creams to experiment with on any rash."

The lesson here? Know your itch first. Then scratch it, or soothe it with the right medicine. ■